Literature review protocol

Annex C | PRISMA-P structure | Farah Hammami | v1.0, 13 November 2026

Title and domain

Mapping evidence on uterine fibroids for an evidence-aware selective RAG assistant. This is a scoping review with a separate targeted technical literature search. The original pilot ran on 12 November 2026; this protocol records the refined plan before formal screening, not before the pilot.

Research question (PCC)

Population: adults affected by uterine fibroids. **Concept:** evidence and applicability of classification, diagnosis, treatment and fertility outcomes. **Context:** international academic literature, with specific attention to Tunisia.

What evidence and conditions of applicability are reported for uterine fibroids in guidelines and research syntheses, including the Tunisian context, that can support an auditable academic question-answering corpus?

Objectives

Identify evidence categories, populations, outcomes, disagreements and missing information. Use them to design passage metadata, answer limits and FibroidQA questions. The separate technical search informs retrieval, verification, citations and abstention.

Inclusion criteria

Medical sources: 1 January 2016 to 12 November 2026; French or English; adults; official guidelines, systematic reviews, meta-analyses and FIGO classification sources. Targeted randomized trials may fill gaps; aggregated Tunisian observational studies may provide local context. Require identifiable methods or provenance, relevant outcomes, and lawful full-text access for extraction. Older foundational sources may be justified separately.

Technical sources: 2023 to 12 November 2026, French or English, RAG/QA studies with explicit methods and evaluation. Foundational 2020-2022 work may be added with a reason. Preprints are labeled separately and never treated as clinical evidence.

Exclusion criteria

Exclude animal/in vitro studies, isolated case reports from the main corpus, acute pregnancy and cancer topics outside scope, unsupported opinion or marketing, duplicates, retractions, inaccessible full texts, unrelated topics and languages outside FR/EN. Exclude patient-level data and technical articles without an explicit method or evaluation from the core set. Keep inaccessible but useful references as pointers only.

Sources and search strategy

PubMed, Europe PMC and OpenAlex were searched through their APIs on 12 November 2026. Planned supplements: FIGO, NICE, ACOG, ASRM, Cochrane, INEAS, Tunisian Ministry of Health, local journals, ACL Anthology and publisher sites. Record dates and URLs when actually searched; do not assign hit counts to unsearched sites. Use backward and forward citation checking, logged separately.

Query	Database	Hits	Exported	Decision
MED-v1	PubMed	5273	30	Broad comparison only
MED-v2	PubMed	317	30	Main medical
MED-v3	PubMed	17	17	Tunisia supplement
MED-v4	PubMed	82	30	Conditional trials
TECH-v1	PubMed	751	30	Comparison only
TECH-v2	PubMed	803	30	Targeted technical
TECH-v3	OpenAlex	51841	30	Broad comparison only
TECH-v4	OpenAlex	599	30	Targeted technical
TECH-v5	OpenAlex	2050	30	Targeted technical
TECH-v6	OpenAlex	3075	30	Targeted technical
TECH-v7	OpenAlex	13188	30	Targeted technical
MED-v5	Europe PMC	164	30	Main supplement

The exact versioned search strings, fields, date filters and query decisions are in review/search_queries.csv; the original API query strings and UTC timestamps are in review/pilot_run_local/log.json. They form the search-equation annex to this protocol. The pilot sampled up to 30 records per query; the medical review will retrieve all available results from retained medical queries. The technical search is deliberately targeted (first 30 per retained query plus citation follow-up), not exhaustive.

Selection

One human reviewer (me) will deduplicate DOI/PMID, check near-duplicates and preprint/published versions, then screen titles and abstracts. Uncertain records proceed to full text. Full-text exclusions receive one main reason. AI may suggest labels but cannot exclude papers or count as a second reviewer. I will calibrate on 10 records and re-code 15% later for intra-reviewer agreement. If a second human is available, record raw agreement and Cohen’s kappa before resolving disagreements.

Extraction

Medical: reference ID, year, country, population, FIGO, study design, intervention/comparator, outcome, effect and confidence interval if reported, limitations, evidence certainty, source section, license, version and applicability to Tunisia. Technical: model, dataset, split, corpus, retrieval, claim checking, metrics, baselines, costs, code and limitations. Use the CSV extraction templates. Mark missing information NR; verify against the full text.

Quality and risk of bias

Use AGREE II for guidelines; AMSTAR 2 for intervention systematic reviews; RoB 2 for randomized trials; appropriate JBI checklists for observational studies and QUADAS-2 if diagnostic accuracy studies are included. Record GRADE certainty when reported, without making a universal score. For technical papers, record data leakage risk, split, baseline fairness, annotation, code availability and costs. Use judgments by domain, not an automatic quality cutoff.

Synthesis

Use a narrative map grouped by evidence type, population, FIGO, outcome, setting and uncertainty. Do not pool heterogeneous clinical effects or compare benchmark scores from incompatible experiments as if they measured the same thing. Create a source-to-question matrix for the corpus and describe gaps.

Design decision and limitations

Choose evidence-aware selective RAG (B), keep standard RAG as the baseline (A), and exclude a graph (C) due to validation cost. Main review risks: publication and language bias, indexing delay, single reviewer, inaccessible text, stale evidence, incomplete Tunisian information and a small technical sample. Mitigate with complementary searches, recorded decisions, re-coding, versioned sources and explicit limits.

Version control and ethics

The pilot refined the query choices before formal screening. Any later change will be dated and justified in Git. No formal study inclusion or clinical validation has occurred. No patient data; only public/authorized evidence and synthetic questions. No external protocol registration is claimed.

Method references

Moher et al., PRISMA-P (2015), doi:10.1186/2046-4053-4-1. Tricco et al., PRISMA-ScR (2018), doi:10.7326/M18-0850. PRISMA 2020: https://www.prisma-statement.org/prisma-2020. These are reporting frameworks, not a replacement for a review method.